

LITTLE BLUE BOOK NO. 689
Edited by E. Haldeman-Julius

Woman's Sexual Life

William J. Fielding

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Author: William J. Fielding

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SEXUAL LIFE ***

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Woman's Sexual Life

William J. Fielding

Author of "The Caveman Within Us," "Sanity in Sex," "Health and Self-Mastery Through Psycho-Analysis and Autosuggestion," "The Puzzle of Personality," "Autosuggestion—How It Works," "Psycho-Analysis—the Key to Human Behavior," "Rejuvenation—Science's New Fountain of Youth," "Rational Sex Series," Etc.

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By William J. Fielding

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WOMAN'S SEXUAL LIFE

CHAPTER I

SEXUAL INSTINCT IN WOMAN

Sexual emotions are normally more powerful in man than in woman.

On the other hand, woman's sexual nature, because of her biological function of motherhood (nature, of course, makes no distinction between actual and potential) is more diversified in its manifestations. It is less dominated by a powerful urge that seeks specific expression, less centered upon the immediate goal of concrete sexual experience, and is instinctively more concerned with the mate as a companion and protector, with children as "flesh of her flesh and blood of her blood," and with the home as a haven of safety.

This not a question of choice or will, but is due to the character of woman's physiological being, with its highly organized nervous system. The peculiar development of her sympathetic organization, which is responsible for her more sensitive emotional nature, also governs the particular tendencies of her love life.

Because of her physiological structure, which is responsible for the psychological reactions peculiar to woman, and not *vice versa*, it is inevitable that this should be so. Sexual conservatism and quiescence are demanded by the responsibilities of motherhood, whether actual or potential. Of course, as our social environment is far from a natural world, there are many modifications in sexual conduct in all its phases. *Basically*, however, woman is sexually conservative, comparatively passive, but nevertheless possesses a sexual organization more highly ramified than that of man, and with different reactions and different ways of manifesting itself.

Man's sexual nature is more centered upon the love object from the standpoint of sexual experience, more ego-centric, more dynamic.

This phenomenon of differentiation of sexual temperament in the two sexes is not peculiar to the human race, but extends throughout the entire animal kingdom. It is epitomized, however inadequately for our purpose, in

the axiom that among all organized beings, the male pursues the female of its choice.

The case of pathologically hysterical women may seem to contradict this contention. But this concerns morbid types, which are exceptional. Incidentally, hysteria tends to prove absence of control of the superior centers which rule the sexual instincts (Clouston). As has been widely observed, the manifestation of a more intense sensuality in woman, or the presence of abnormal eroticism, coincides with the appearance of criminal tendencies.

In Messalina and Agrippina, (the mother of Nero), and countless others less classical, the excessive life of the senses has been associated with no little ferocity. These types seem to be congenital criminals, irresponsible, though often brilliant. In all of the women of this category, as evidenced by numerous examples cited by Lombroso and Ferrero, the vehemence of the passions even begins to assert itself before puberty. Their social life commences with the dawn of sensuality. They have a stronger inclination toward the masculine pattern, and a more intense curiosity than normal women. Probably for the same reason they accommodate themselves more easily to polygamy.

WOMAN'S EMOTIONAL NATURE

It has not been sufficiently emphasized that in woman the sympathetic or involuntary nervous system, which controls the vegetative and sensitive life, is more highly developed than in man. This is a physiological necessity, as woman has additional organs, the breasts, womb, ovaries, and associated organs not existing in man. As woman has more organs concealed in the pelvis, with a greater complexity of function, her sympathetic nerves are necessarily more numerous.

It is mainly due to this elaborate sympathetic system that women feel more keenly the emotional side of life. They are more subject to fear, shock, and fright, and more easily aroused to joy or sorrow. Their affections are deeper, and their grief more intense.

The sympathetic nervous system regulates the tears that are shed in grief, the salivary and gastric secretions that are checked in fright, as well as the

milk-flow that is arrested, increased or altered under the influence of terror, or maternal love, as the case may be.

It is also due to the influence of this sympathetic system over the calibre of the blood-vessels that women blush more readily. Moreover, blushing is most pronounced at the time that the pelvic organs begin their rapid development with the onset of puberty.

The ancients attributed hysteria, a nervous and mental disturbance peculiar to women, to causes within the womb. And undoubtedly there is a connection; not that the disorder is actually seated in the pelvic regions, but these organs form part of a reflex arc to the brain centers through the sympathetic system. In hysteria the system is probably in a state of hyperesthesia, or over-sensitiveness.

It is also significant to note that while woman's brain is narrower than that of man, and smaller in the motor centers, therefore exhibiting less energy and force, it is more voluminous in the posterior part. In other words, women are more long-headed than men, and it is the posterior area that is concerned with the manifestation of the feelings and emotions which are stronger in women.

Furthermore, this posterior part of the brain (the somatic sensory area of Flechsig) is related by means of the sympathetic nervous system with the organs of the chest and abdomen. Again, another connecting link with the emotional life, as we know of the relationship of the heart action and respiration, as well as organs of the abdomen, to feeling and the emotions.

The feminine sex is pre-eminently the affectionate sex. Physical excitements provoke reflexes more readily and more strongly in woman. Her emotionalism is more demonstrative under all circumstances than man's.

This tendency, of course, has its destructive and anti-social as well as constructive and social sides. Among uncultured or primitive women—including modern women in whom the primitive impulses dominate—their emotional lengths lead them to extremes that few men can equal. It has been observed that in countries where enemies are tortured, the women surpass the men in cruelty.

Among the Dakota Indians, old men captives were delivered up to the women who took special pleasure in subjecting them to all sorts of tortures. Bertillon has presented many proofs of the same kind borrowed from the life of other savage peoples. Lombroso instances terrible examples of this kind of ferocity in women who had taken part in political revolution, as well as in women of the criminal world.

On the other hand, we also see feminine sympathy and helpfulness reach the noblest proportions even when, perhaps, it may be altogether unlooked for. Among the same savages who break out intermittently in acts of barbarism, travellers and explorers have cited numerous instances of gentleness and goodness. Stanley, Livingstone and others have attested to this trait.

STRENGTH OF SEXUAL IMPULSE

While the strength of the sexual impulse in woman is subject to a wide range of variation, there is no doubt in the mind of the serious student of sexual phenomena that its normal manifestation is quite invariably a factor to be reckoned with.

In modern times, outside the rational sex educational movement, the notion has prevailed that the sex impulse in woman, at least in “good”, “respectable” women, is a negligible quantity. She was never supposed to evidence any sexual feeling, erotic desire or passion.

Countless women celibatists in the Church and pious sisters in the cloisters have given conclusive evidence, by their actions and lives, notwithstanding their asceticism and religious exaltation, of the potency of the sexual instinct.

The cloister-rules of the Middle Ages prescribed venesection for nuns as well as monks, for the purpose of controlling the sexual urges. Among other less drastic measures employed were narcotics and fasts. Certain cloisters, in order to reduce temptation, have not permitted women to enter their ranks before the age of forty years.

Max Reinhardt’s elaborate version of “*The Miracle*”, which has had such a tremendous vogue in recent years, both in Europe and America, is based on a legend dealing with the irrepressible desires of the flesh. If the

religious devotee who witnesses this spectacular pantomime is entirely lost in the magnificent pageant of a medieval religious atmosphere and ritual, certainly the psychological student is no less carried away by the realistic portrayal of suppressed desires, at times highly symbolized, at other times stark and unmasked. The erotic nature of the Nun's promptings and feeling are set forth as faithfully as Freud himself could have done it. It is a masterpiece in depicting the conflicts engendered between natural desires, or biological impulses, on the one hand, and ideals based on repression of these impulses on the other. The sensuous body of *Megildis*, the Grey Nun, unconsciously revolted against the decree of a mind dedicated to celibacy and austerity—and the desires of the body triumphed while they lasted. After seven years (note the mystical “seven” which appears and reappears in all religious, mythological and legendary forms) the human appetites are appeased, and the Nun returns repentant to the cloister.

Even the act of ultra-fervent devotion has its significance to the initiated. Maudsley has emphasized the influence of physiological conditions upon religious feeling. He shows in this connection that the ecstasy of such devout women as Catherine of Sienna and the holy Teresa, when they imagined themselves visited by Christ, who “embraced each of them as His bride”, was nothing but a vicarious sexual orgasm. This sort of expression, caused by the sight of the nude male figure reproduced in all its normal proportions on the cross, is a more natural consequence among young women of a sensitive and nervous temperament than is generally realized.

In the Middle Ages, the sexual impulse normal to womanhood was perhaps better appreciated than it has been in later times. Luther gave utterance to the opinion prevalent in his day when he wrote: “It is just as impossible for a woman to do without man as for a man to do without woman.” In his treatise “*About Married Life*”, he expressed himself in this candid fashion: “If a potent wife has an impotent husband and cannot openly procure another, and does not for the sake of her honor wish to do it, she should address him as follows: ‘My dear husband, you are my debtor and have deceived me and my young life and endangered my honor and eternal welfare; before God we are not married; allow me therefore to enter a secret marriage with your brother or nearest friend.’” It is the husband's duty, Luther maintained, to comply with her demand, and if he does not wish to do so, he must not be angry if his wife should run away from him.

(K. Hagen: Deutschlands lit. u. relig. Verhältnisse im Reformationszeitalter.)

Luther also summed up the subject, as a natural phenomenon, in these words: "He who wishes to restrain the impulse of nature and not to allow it free play, as nature will and must, what does he do but this: to insist that nature shall not be nature, that fire shall not burn, that water shall not be wet, that man shall neither eat, drink, nor sleep."

Buddha, who fought out the conflict between desires and the will to renunciation, and therefore ought to know about it, delivered himself in this wise: "The sexual impulse is stronger than the ankus with which the wild elephant is controlled, it is hotter than flame, it is like unto an arrow driven into the spirit of man."

Mantegazza stresses the point that in the female sexual desire is very rarely accompanied by pains analogous to those which occur in man, in whom sexual excitement manifests itself in painful tension of the testicles and the seminal vesicles, or in spasmodic, long-continued priapism.

Sergi, with somewhat hectic imagination, communicated his thoughts on the subject to Lombroso, as follows: "The normal woman loves to be flattered and wooed by man, but yields to his sexual desires only like an animal at the sacrifice. It is well known how much pains must be taken, how many caresses must be expended, before a woman will yield with pleasure to a man's desires, and will share his sexual passion. Without the employment of these means, a woman remains cold and gives as little satisfaction as she feels. There are girls who are quite obtuse to the joys of love, and either resist energetically a man's approaches, or yield to him passively, without ardor and without enthusiasm. It is well known, also, that among the lower races of mankind, means are employed to stimulate sexual sensibility in women, means that seem to amount to torture; and that the male with the same end in view, undergoes the most painful operations, from which it is apparent that the slight sexual sensibility of women in these lower grades of civilization is fully recognized." And again, "If a normal woman marries for love, she hides that love deep in her heart; she often complains later that in her husband the love-fervor of the first days still continues; the very moderate sexual needs of the wife form a natural and most valuable check to the much more powerful passion of the male."

Considering woman's function in the realm of sex, her relative conservatism is based on very fundamental grounds. As Professor Erb has pointed out, woman is the principal sufferer in the process of reproduction. For the most part, with a cruelty that baffles the processes of the rational or philosophic mind, nature has condemned woman to a far more difficult position than man in the intercourse of the sexes and in the preservation of the species. In her biologic role, she has been overpowered and forced by man. She is always compelled to make the most severe sacrifices for the sake of the new generation, first when the new being is germinating within her womb, later in the throes of parturition, and finally when the helpless mite of humanity is entrusted to her maternal care. And only too often she fails to receive the respect and protection due to her for the performance of these difficult functions. When everything is considered, it is fortunate that as a rule the young woman who has never come into intimate contact with the male appears to be endowed with a relatively weak sexual urge.

The sexual impulse in normally constituted individuals, as Kisch states, is by no means constant in its intensity. Apart from the temporary indifference resulting from sexual gratification, and apart from the decline in the impulse that sometimes occurs after prolonged continence, the mode of life is a factor of very great influence. The urbanite who is continually reminded of sexual matters, and subjected to greater erotic stimulus, is quite inevitably more disposed than the countryman to sexual excitement. A sedentary and sheltered mode of life, a diet composed too largely of animal food and highly seasoned condiments, have a stimulating action on the sexual life.

In the woman, the sexual impulse is stronger just after menstruation, and during the period it may also be more than usually pronounced. In neuropathic women this increase of excitement may occur in a pathological degree.

Sexual desire in woman, it is generally agreed, outlasts the active sexual life; that is, the erotic impulse continues to manifest itself after the cessation of menstruation, when the possibility of conception has passed. It appears, therefore, that sexual desire is not necessarily dependent upon ovulation.

Probably no other individual has written so extensively and at the same time so wisely of sex matters as Havelock Ellis, who speaks of the

differential characters of the sexual impulse in the female as follows:

“In courtship, woman plays a more passive part than man; in woman the physiological mechanism of the sexual processes is more complicated, the orgasm develops more deliberately; the sexual impulse in woman needs more frequently to be actively stimulated; the culmination of sexual activity is attained later in life than in the life of man; the strength of sexual desire in woman becomes greater after she has entered upon regular sexual intercourse. Women bear sexual excesses better than men; the sexual sphere is larger and more widely diffused in women than in men; finally, in woman the sexual impulse exhibits a distinct tendency to periodic exacerbations (malignity or violence of disease), and it is in any case much more variable than in man.”

Ellis further maintains that the source of erotic pleasure in the case of the male lies in activity, but in the female in the passive state, in the experience of loving compulsion, as it were, and he holds that sexual subordination is a necessary element in the sexual enjoyment of women.

CHAPTER II

FACTORS THAT INFLUENCE SEXUAL LIFE

Conspicuous among the factors that influence woman's sexual life for good or ill, of course, is marriage. When the marriage is everything that it ought to be, particularly when both parties, and especially the husband, possess a happy insight into the problems of sex, there is at least assured to the wife the foundation for a healthy amative life. And so much depends upon this factor that it alone may be considered the bedrock not only of marital harmony, but of the woman's physical, mental and spiritual well-being. That the man's destiny also is inescapably bound up with these conditions is, of course, self-evident.

Proper sexual gratification—meaning regular indulgence of this urge, free from excesses—fulfills a need that has no real substitute in the life of the normally constituted woman. This is true even though some authorities may have over-emphasized the drawbacks of sexual abstinence. As a matter of fact, many women have overcome the problems of a celibate life with seeming serenity, and certainly with tremendous achievement to their credit. On the other hand, countless numbers have found it an insurmountable handicap, sometimes harboring disastrous results. So much depends upon the hereditary constitution, the bodily chemistry, and temperament of the individual, and also upon the environmental influences that have helped to mould the person's life!

Often the inability to marry will make a deep impression on the mental life of woman, and in many cases lead to passionate desires and tormenting yearnings of an erotic nature. If promiscuous indulgence is resorted to, to relieve the distress, this generally leads to conflicts with the ethical ideals, or at least to distressing pangs of conscience from the realization that the social conventions are being defied.

In addition to this prolific source of mental disturbance, there is the possibility of physical impairment as a result of venereal diseases, or even of pregnancy, from the illicit relations.

Only too often occasional promiscuity leads to confirmed prostitution, which as a world-wide phenomenon is believed to be on the increase, even if segregated districts are on the decline in American communities.

Among the reasons attributed to the increase in prostitution are the growth of modern industry—particularly in countries formerly principally agricultural—with the consequent aggregation of population in the large towns and cities, the decline of the marriage rate, the necessity for the postponement of marriage, and universal military service in many countries.

In any event, while the sexual impulse is a factor in promiscuity, and the pathological sexual constitution is a factor in prostitution, it cannot be gainsaid that the economic motive—hunger, and the desire for better dress and adornment (in itself not an aspiration to be condemned)—is the greater incentive to this perilous life.

The world over, the poorest paid classes of workingwomen are shown by statistics to furnish the largest number of recruits to the ranks of prostitution. And during periods of unemployment and hard times, it is notorious that the number of women practicing occasional prostitution is greatly increased.

To recur to the subject of marriage, the question of the ages of the parties to the union is an important one. While there are exceptions to all rules, it is safe to say that, other things being equal, it is better that too great a discrepancy should not exist in the ages of man and wife.

In this connection, Kisch states: “Not only is the absolute age of the woman of importance in deciding on the advisability of marriage, but the relative ages of the proposed husband and wife must also be taken into account, first of all in respect of the wife’s possible fertility, and secondly in respect to her general health. The most suitable arrangement is that in which there is no marked difference in age. The husband may, and indeed in existing social circumstances almost necessarily is, somewhat older than his wife, as much perhaps as eight or ten years. But a very great disparity of age (in either direction) is a serious error. If a very young girl marries an elderly man, or a developed matron marries a young man, the true purpose of marriage is unfulfilled, the eternal laws of nature and all ethical principles are infringed. In the breeding of animals, the fundamental principle has long prevailed that the animals chosen for coupling should be

well suited each to the other and should be in perfect physical condition; and breeders are also familiar both with the favorable influence of good environment and with the opportune crossing of distinct varieties. The same principles are equally applicable to the human race, neglected as they commonly are in practice.”

In several other of the Little Blue Books, (Nos. 653 to 657 inclusive) of this Rational Sex Series, I have discussed certain problems relating to marriage which inevitably intrude themselves, so it will be unnecessary to continue the discussion here. Among these questions—for the benefit of those who have not read the previous books—are: Frequency of Sex Relations, Intercourse During Menstruation, Intercourse During Pregnancy, Intercourse for Reproduction Only, Reciprocity in the Sex Relations, Overcoming Sexual Coldness, Wooing as a Psychic Preparation, Embryology, Hygiene of Pregnancy, etc.

Suffice to say that sexual hygiene entails a certain moderation in the physical expression of love, and also a certain constancy, as may be expected in a happy marriage. The wife acts wisely and in accord with her normal role in the sexual sphere in not acceding at once and unconditionally to her husband’s amatory advances leading to a repetition of the marital relations.

Modest reluctance and hesitancy on her part, with a prolonging of the love play which naturally follows, perfects the mental and spiritual state for the acme of enjoyment to be derived, as well as enhances the physical preparation for the act.

SEASONAL AND CLIMATIC FACTORS

There is no doubt that seasonal and climatic factors exercise a marked influence over the sexual development and erotic nature of women. The duration of sexual activity, for instance, is less in the women belonging to the countries of southern Europe than in those belonging to the countries of northern Europe. The data at hand indicate that in those climes in which ovulation begins sooner and menstruation first appears at an earlier age, the menopause or change of life also makes its appearance earlier; and that in those climates in which puberty is late in coming, the decline of sexual activity is similarly postponed.

In the mid-European climates, where puberty makes its appearance in girls at a comparatively early age, the first menstruation occurring between the ages of thirteen and sixteen—corresponding with the time of puberty of girls of the North American temperate zone—there is exhibited a more prolonged duration of the sexual life, of menstrual functional activity, than in the northern women in which menstruation begins late, between the ages of seventeen and twenty.

Extremely early appearance of the first menstruation—so early as to be altogether abnormal—has the same significance as the abnormally late appearance of menstruation. Both indicate that the sexual life will be of short duration.

It is observed, too, that in certain months of the year a definite increase in the number of conceptions continues to recur, which indicates that during these months a larger number of sexually mature individuals is more effectively using the sexual functions.

Some authorities have regarded this as the manifestation of a “physiological custom,” imminent in the physical constitution of civilized man, and inherited by him from his animal ancestors.

Rosenstadt explains the phenomenon in the following manner: “Primitive man inherited from his mammalian forefathers the peculiarity of reproducing his kind only during a certain definite period, the period of heat or rut. After humanity had entered upon this period, copulation was effected en masse, as was easy in view of the primitive community of sexual intercourse before the origin of marriage. In the course of his progress toward civilization, however, man began to reproduce his kind indifferently throughout the entire year, but the original physiological custom, in accordance with which reproduction occurred at definite seasons only, did not disappear, and persists, indeed, to a certain extent, even to the present day, as a survival of earlier mammalian life, and manifests itself in the annual recurrence in certain months of an increase in the number of conceptions.”

“The analogy in structure and function between the genital organs of the human species and those of other mammals (the female anthropoid apes do not merely exhibit from time to time a period of heat, but are subject to a more or less regular menstruation), which for the most part reproduce their

kind only at certain definite periods, leads to the conclusion that in the human species also the sexual impulse may originally have awakened only at a particular season of the year, and that the persistence of this physiological custom in man, in spite of the fact that sexual intercourse occurs all through the year, and notwithstanding that the conditions necessary to awaken the sexual impulse are actually perennial, must be ascribed to inheritance.”

Other sexologists and biologists share this view, which finds support in ethnological investigations, and deductions derived therefrom, particularly as set forth by Kulischer and von Hellwald, who reasoned that the sexual relations of primitive man took place only at certain seasons, namely at spring and at harvest-time. This theory receives added probability from a number of actual and symbolical practices among different races.

MORTALITY OF MARRIED AND UNMARRIED WOMEN

A comparison of the mortality of married and unmarried women at different ages furnishes some significant observations. It is readily understood that the full evolution of the sexual life brings in its train the possibility of many dangers to a woman's life. I have already commented upon the heavier burden which Nature has placed upon woman.

Between the ages of 20 and 25 years, the mortality of married women is in all races higher than that of unmarried women. The same holds true between the ages of 25 and 30 years, excepting in France, in which country the general practice of birth control methods causes maternity to cease at an early age, and thereby reduces the mortality tolls in this respect.

Kisch states that in Prussia, in a certain year, of every 10,000 married women between the ages named above, 21 died; of every 10,000 unmarried women, only two died.

In Holland, Belgium and Bavaria, this excess in the mortality of married women continues up to the age of 40 years; while in Prussia, from the age of 30 upward, the mortality of married and unmarried women is practically the same. In many countries, the mortality of married women at many ages exceeds even that of unmarried men.

On the surface, this would appear that marriage is a comparatively unhealthy institution to womanhood. Further reflection will show, however, that it is not primarily marriage in itself that jacks up the mortality of women during this period of greatest sexual activity.

There is every reason to believe, as well as incontrovertible evidence to prove, that regular, normal satisfaction of the sexual impulse is beneficial both to men and women, conducive to health, well-being and long life.

The young married woman, it must be remembered, is in a countless number of instances exposed to venereal infection, which the husband has carried over from his bachelor days (or nights). Then there is not only the question of mortality directly from venereal diseases to be considered from the woman's standpoint, but the many secondary ailments or the undermining of the constitution that result from gonorrheal or syphilitic infection of the system. Any number of diseases which do not show this contributory factor in the mortality statistics have their genesis in, or receive an impetus from, venereal infection.

Furthermore, a woman whose system has been weakened, or her blood poisoned, or certain of her vital organs crippled by attacks of the *gonococcus*, or the *spirochaeta pallida* of syphilis, suffers an additional handicap in pregnancy, parturition, and the subsequent stages of the puerperium.

There are, of course, also the domestic problems, many of which have their root in unfavorable economic conditions—trying to run a household on an insufficient budget. This is especially a hardship when there are several children to feed, clothe and educate on the salary or wage of the average man of our period. A situation of this kind to a sensitive woman, with ambitions for the future of her children, and a thought of the prospects of herself and husband in old age, is apt to lead to depression and nerve exhaustion in many cases.

The excessive mortality of married life, however, is chiefly due to venereal pathology, and in a lesser degree to cases of sexual ineptitude on the part of the husband. In regard to the latter, Kisch states: "Even coitus, in cases in which there is a great disproportion in size between the penis and the vaginal orifice, or when the organ is very rapidly introduced or the act is

very roughly performed, may lead to injury to the vulva or the vagina, a fact to which a very large number of recorded cases bears witness.”

The lower mortality of married women above forty years of age, as compared with unmarried women at the same period of life, indicates the advantage to the former of the fulfilment of the sexual functions in the normal manner.

SOME ASPECTS OF PREGNANCY

While conception during any period of the month is known to be possible, it is generally agreed among authorities that the first days after menstruation are the most favorable for conception.

Statistical data covering a fairly wide scope show that as the date of the next menstruation period is approached, there is a continual decline in the frequency of conception. Just before the flow it rarely occurs.

Dr. Hensen, a reliable European authority, draws the following conclusions from the record of 248 cases in which the date of the fruitful intercourse was exactly known:

1. The greatest number of conceptions follow coitus effected during the first few days after the cessation of the menstrual flow.
2. When coitus is effected during menstruation, the probability of conception increases day by day as the end of the flow is approached.
3. The number of conceptions following coitus effected shortly before menstruation is minimal.
4. However, there is no single day either of the menstrual period or of the inter-menstrual interval, on which the possibility of the occurrence of conception can be excluded.

In human beings it is extremely difficult to tell the exact date of conception, if intercourse has been indulged in with more or less frequency, as is usually the case. Even when the date of coitus has been noted, if a specific act can be attributed to conception, there is no telling when

fertilization takes place. The spermatozoon may not meet the ovum for several days, during which interval the male fertilizing element remains alive in the woman's sexual organs. The longest survival of human spermatozoa so far known is three and one-half weeks.

The duration of pregnancy is nine calendar, or ten lunar months—or from 275 to 280 days. It is usually calculated as 280 days from the beginning of the last menstruation. As the expectant mother naturally wishes to know as nearly as possible the date of confinement, a table is shown on pages 26-27, which will be helpful for this purpose.

If the pregnant woman knows the day her last menstruation began, she can tell in a moment when confinement may be expected. There may be a variation of a few days, one way or the other, on account of the uncertainty of the time of fertilization—but the table will be found serviceable for general use.

The onset of pregnancy represents the acme of a cycle of profound changes in woman, with a wide range of emotional and psychic reactions.

Stratz, a competent authority as regards the subject of feminine beauty, affirms that a beautiful woman is most beautiful when the period of maximum beauty coincides in her case with the first month of her first pregnancy.

With the commencement of pregnancy, the processes of nutrition are accelerated, all the tissues are tensely filled, the skin is more delicately and at the same time more brightly tinted on account of the greater activity of the circulation, the breasts become firmer and more elastic. Thus the attractive characteristics of beauty at its fullest maturity become enhanced. This bloom lasts for a short time only, however, since the enlargement of the abdomen in the further course of pregnancy impairs the harmony of the figure, and the increasing demands of the embryo upon the woman's own system detract from her normal appearance.

While pregnancy is, of course, a natural physiological process, and in most cases is concluded without untoward result, it nevertheless makes a deep impression on woman's entire nervous organization. This is especially noticeable in the case of first pregnancy. It is perfectly obvious why this

should be so, as the woman is filled with expectation and anxiety concerning an unknown event.

TABLE FOR CALCULATING THE DATE OF CONFINEMENT

Jan.	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
Oct.	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23
Feb.	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
Nov.	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23
Mar.	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
Dec.	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21
Apr.	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
Jan.	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21
May	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
Feb.	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20
Jun.	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
Mar.	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23
July	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
Apr.	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22
Aug.	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
May	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23
Sep.	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
Jun.	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23

Oct.	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
July	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23
Nov.	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
Aug.	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23
Dec.	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16
Sep.	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22

(Table continued)

17	18	19	20	21	22	23	24	25	26	27	28	29	30	31	Jan.	
24	25	26	27	28	29	30	31	1	2	3	4	5	6	7	Nov.	
17	18	19	20	21	22	23	24	25	26	27	28					Feb.
24	25	26	27	28	29	30	1	2	3	4	5					Dec.
17	18	19	20	21	22	23	24	25	26	27	28	29	30	31	Mar.	
22	23	24	25	26	27	28	29	30	31	1	2	3	4	5	Jan.	
17	18	19	20	21	22	23	24	25	26	27	28	29	30			Apr.
22	23	24	25	26	27	28	29	30	31	1	2	3	4			Feb.
17	18	19	20	21	22	23	24	25	26	27	28	29	30	31	May	
21	22	23	24	25	26	27	28	1	2	3	4	5	6	7	Mar.	
17	18	19	20	21	22	23	24	25	26	27	28	29	30			June
24	25	26	27	28	29	30	31	1	2	3	4	5	6			Apr.
17	18	19	20	21	22	23	24	25	26	27	28	29	30	31	July	
23	24	25	26	27	28	29	30	1	2	3	4	5	6	7	May	
17	18	19	20	21	22	23	24	25	26	27	28	29	30	31	Aug.	

24	25	26	27	28	29	30	31	1	2	3	4	5	6	7	June
17	18	19	20	21	22	23	24	25	26	27	28	29	30		Sep.
24	25	26	27	28	29	30	1	2	3	4	5	6	7		July
17	18	19	20	21	22	23	24	25	26	27	28	29	30	31	Oct.
24	25	26	27	28	29	30	31	1	2	3	4	5	6	7	Aug.
17	18	19	20	21	22	23	24	25	26	27	28	29	30		Nov.
24	25	26	27	28	29	30	31	1	2	3	4	5	6		Sep.
17	18	19	20	21	22	23	24	25	26	27	28	29	30	31	Dec.
23	24	25	26	27	28	29	30	1	2	3	4	5	6	7	Oct.

NOTE.—Find in top line the date on which last menstruation commenced. The figure under it will indicate when confinement may be expected. If menstruation commenced January 20, confinement may be expected October 27, or one day earlier if leap year.

There is a virtual revolution going on within her organism, producing powerful impressions on her mental processes, and setting up currents of widely ramified physical reactions. Joyful hopes are cherished, distressing fears are felt, while automatically within her own being there is transpiring the physical aspect of creation.

CHAPTER III

FUNCTIONAL PERIODICITY—

MENSTRUATION

Menstruation, or the monthly flow of blood from the genital organs, which manifests itself in the human female from the time of puberty, is but one evidence of a periodic excitation that affects her whole organism.

The phenomenon particularly reacts upon the entire nervous system, blood-vascular system, and internal chemistry, and through these processes, all the organs and tissues feel the impress of the periodic movement.

The menstrual period in itself, as has been recognized in more recent years, is but the climax of a monthly physiological cycle which influences throughout the month the whole of woman's physical and psychic organization. As Havelock Ellis has stated, whatever organic activity we investigate daily with any precision, we find traces of this rhythm. "While a man may be said, at all events relatively, to live on a plane, a woman always lives on the upward or downward slope of a curve. This is a fact of the very first importance in the study of physiological or psychological phenomena in women. Unless we always bear it in mind we cannot attain to any knowledge of the physical, mental or moral life of woman."

It is perhaps natural that reference to the incident of menstruation in the life of woman should be found as far back as there exist records of human speculation. The very earliest ideas on the subject were based on the assumption that menstruation was an indication of woman's inferiority, and stigmatized her as "unclean."

These notions generally were due to man's lack of knowledge concerning this process of periodicity. The closely interrelated superstitions, taboos, mythological concepts and religious ideas of primitive and ancient times caused the moral leaders to dogmatize over the subject, to the extent that women are still hampered by the traditions that began at a very ancient, and possibly prehistoric, period.

Thus in the fifteenth chapter of Leviticus, 19-30, we are told: "And if a woman have an issue, and her issue in her flesh be blood, she shall be in her impurity seven days; and whosoever toucheth her shall be unclean until the even. And everything that she lieth upon in her impurity shall be unclean. Every bed whereon she lieth all the days of her issue shall be unto her as the bed of her impurity. But if she be cleansed of her issue, then she shall number to herself seven days, and after that she shall be clean. And on the eighth day she shall take unto her two turtle doves, or two young pigeons, and bring them unto the priest, to the door of the tent of meeting. And the priest shall offer the one for a sin-offering, and the other for a burnt offering; and the priest shall make atonement for her before Jehovah for the issue of her uncleanness."

This feature of the Mosaic law was also followed by the Mohammedans, who likewise regarded a menstruating woman as unclean, and who remained unclean for a certain number of days after the flow ceased, or until she had been cleansed by ecclesiastical influence.

It is to be expected, of course, that the earliest medical writings, under the social and religious influence of the times, should also evince a similar feeling toward the subject. The early Indian book of *Suśruta*, and the subsequent writings of Hippocrates, gave evidence of this attitude.

Many savage races regard the first appearance of the menstrual flow as the only certain sign of puberty, and among them this is the occasion of peculiar ceremonial rites. The advent of puberty in savage tribes is often solemnized by the seclusion of the girls from the time of the first menstruation. They fast during the period of seclusion, which sometimes terminates in an elaborate ritual of purification. (Thus, we again meet the old belief of woman's innate uncleanness.)

FIRST APPEARANCE OF MENSTRUATION

As we have observed, climate has a definite influence on the sexual development of woman, and therefore on the age when menstruation begins, signifying the beginning of puberty. In the torrid zone, menstruation appears at a very early age, on the average from 11 to 14; in the temperate zone it appears later, on the average from 13 to 16; and in the frigid zone later still, on the average from 15 or 16 to 20. The altitude above sea level

and the distance from the coast are also known to have a certain influence. These factors are probably caused, to an extent at least, by the scarcity of iodine in its natural food form in high altitudes and particularly away from the coast. In salt water fish and marine foods this important element, which influences the bodily metabolism, is present in liberal quantities, while in all the foods prevalent away from the coast and in mountainous regions, there is a lack of natural iodine that often has an injurious effect upon the system. For some reason or other, women seem to be the greater sufferers from this deprivation, and the incidence of goitre is often traced to this cause.

Race and constitution also have a decided influence upon the appearance of puberty. In women of the Semitic races it occurs earlier than in women of the Aryan races. The average age at which menstruation begins in Jewish girls is from 14 to 15; in Magyar (Hungarian) girls from 15 to 16; in girls of the Germanic races, from 16 to 16-1/2, and in the Slavonic girls from 16 to 17 years.

Generally speaking, puberty manifests itself earlier in girls of a sanguine, lively temperament and a strong constitution than in those of a phlegmatic temperament and a weakly constitution. Furthermore, other things being equal, menstruation appears earlier in brunettes, girls with black hair, thick skin, dark eyes and a dark complexion, than it appears in blondes, girls with light hair, thin skin, blue eyes, and a fair complexion.

Social circumstances and conditions of life are also influencing factors. According to Kisch, in the higher classes of society, and among the well-to-do, whose mode of living is luxurious, and whose social circumstances allow free play to the imagination, menstruation appears earlier than among women of the laboring classes who are compelled to exert themselves for a bare living.

In cities and large towns, too, menstruation is observed to appear earlier than in the small towns, while in the open country it is still further delayed.

The initial appearance of menstruation is commonly preceded by various symptoms, among which are: A dragging sensation in the loins; an indefinite feeling of pressure in the lower part of the pelvic regions, especially in the neighborhood of the uterus and the ovaries; alternating sensations of heat and chilliness, sometimes accompanied by actual though

slight change of temperature. Not infrequently there are also disturbances in the intestinal evacuations and urinary secretion, and in the functional activity of the gastro-intestinal canal. A commonly observed symptom is an increased irritability of the entire nervous system, with a tendency toward melancholy and indefinite amorous desires.

The nervous irritability usually asserts itself before the incident of the menstrual flow by headache and moodiness, nervous irritability, and low spirits. There may be further slight changes in the facial aspect, dark rings around the eyes, spontaneous blushing, uneasy sensation, loss of appetite, a sensation of pressure in the abdomen, palpitation, vertigo, feeling of weakness and numbness in the lower extremities, etc.

Courty enumerates the following symptoms which are observed in the majority of girls before the first appearance of menstruation: Swelling and tenderness of the breasts, sensation of fullness and weight in the gastric region, gas in the intestines, a watery-mucous vaginal discharge, finally, an itching sensation in the genital organs.

These manifestations may also assume a morbid character, taking the form of violent abdominal or pelvic pains, general fatigue and weakness, dyspepsia and diarrhea, various kinds of neuralgia, and sometimes a degree of moral aberration.

After the first menstruation, two or three months may elapse before the girl menstruates again, but after a year the flow usually recurs at quite regular periods. Sometimes the early periods are very violent and happen with more than ordinary frequency, every twenty days, for instance.

It is interesting to note that some traces of a menstrual period are found among the higher mammals, such as the mare and cow, and also the bitch. Among monkeys and apes in their wild state there is a well-marked menstrual discharge.

In the higher apes, when they do not suffer from captivity, the flow is said to be sometimes quite as copious as in women. Some species of monkeys become swollen and brilliantly pigmented, and give other characteristic evidences of their condition.

As a general rule, however, menstruation, in its fully developed form, may be considered a human characteristic. Not only is the flow more

marked as the mammal approaches Man, but among the lower races of human kind it is less pronounced than among the higher races. Dr. Holder has found, for instance, that the American Indian women menstruate for two days only.

Ellis remarks that in the early history of the race menstruation can but rarely have occurred; the greater part of woman's sexual life having been taken up by pregnancy and very prolonged lactation. Pinard observed that, as menstruation is the expression of the abortion of an unfertilized ovum, "under natural conditions a healthy woman would never menstruate." *Natural conditions*, of course, imply that woman would always either be in a pregnant state, or nursing a child, so that between the two, there would be no opportunity to menstruate.

SYSTEMATIC CHANGES DURING MENSTRUATION

I have remarked that the menstrual period is but the climax of a monthly physiological cycle. However, this climax has more far-reaching results in some women than in others.

Sensitive women, with high-strung nerves, or more particularly of unstable organization, are apt to feel the effects in a very pronounced manner. But even normal, healthy women, under conditions of modern life, experience the reactions to a greater or lesser degree.

Kisch, after citing the purely pathological cases, states: "Finally, we may have to do with isolated sporadic occurrences in which the exciting influence of menstrual processes may be traced. Thus, for the outbreak of a periodical menstrual psychosis (insanity), an especially temporary predisposition must exist, connected with the great development epoch of the sexual life.

"There is, for instance, a group of transitory states occurring during menstruation, and taking the form of disorders of the intelligence or of explosive emotional states; such may be witnessed, not in those suffering from psychopathic predisposition, but in quite healthy individuals."

Menstruation throughout the course of the sexual life may, like the first menstrual flow, but with less intensity, give rise to manifestations of nervous and mental disorders. Many women who are otherwise healthy,

suffer during menstruation from nervous irritability, ill-temper, low-spiritedness, neuralgia, and even hysterical and epileptic attacks. These disturbances occur chiefly on the first and second days of the flow, and disappear toward the end of the period.

The manifestations are more extreme in individuals weakened by inordinate losses of blood, or by deep-seated disorders in various organs. They are also severe in those predisposed to such disturbances from neuropathic inheritance, and more severe in women chronically troubled with painful or excessive menstrual flow.

In his work on the influence of the so-called menstrual wave on the course of mental disorders, Schule observes that the mental equilibrium even of a perfectly healthy woman is not a stable one, but is subject to a series of oscillations. The menstrual period, he continues, has a distinct influence on woman's mental equilibrium.

Even in those who are possessed of a healthy nervous system, menstruation evokes a state, now of depression, now of excitement; in neurotic women on the other hand, menstruation may give rise to nervous diseases which may equally exhibit the characteristics of depression or the characteristics of excitement.

In nervously predisposed women, the influence of regularly established menstruation, even when the circumstances are favorable, is pretty much the same as the influence of menstruation when it first makes its appearance; the influence is merely somewhat weaker insofar as the woman has learned to endure and to be patient. The menstrual state, in nervously predisposed women evokes the particular neurosis to which the individual happens to be liable. The disorders most commonly met with in this association are hysteria, hemicrania (neuralgia on one side of the head), swimming of the head, epileptic paroxysms, toothache, and neurasthenia.

The characteristic form of neuralgia is especially frequent. Sometimes it may begin a day or two before menstruation, and may accompany its whole course, becoming however, less severe toward the end of the flow. Hysteria most commonly manifests itself on these occasions by a depressed emotional state, by tearfulness, by complaints made without sufficient grounds, sometimes by muscular spasms, but very rarely by epileptic seizures.

There are also numerous other evidences of variation from the normal state, but not observable to the untrained eye, or in many instances without special tests. For instance, Cullen discovered long ago that the pulse rate rises at the approach of the periods. Stephenson found a monthly cycle with respect to arterial tension and pulse rate. Giles' investigations showed the greatest blood pressure on the first two days of menstruation and the day preceding.

Thirst is often present, and urination more frequently than usual. There is also a tendency to pigmentation; the pigmented circle around the nipple usually becomes darker, the complexion is changed, losing its clearness, and a dark ring may sometimes be perceptible around the eyes.

According to various authorities, the temperature is at its highest point a few days before actual menstruation. Silva makes the interesting observation that the alkalinity of the blood is reduced during the period, and the vasomotor system reacts to stimuli in the same way as in states of fever.

Havelock Ellis thinks there is little doubt that a daily examination of any of the senses would show a monthly rhythm. He is only acquainted with one series of observations on this point, namely those of Finkelstein of St. Petersburg, who studied the functional activity of the eye during menstruation in twenty healthy women, aged between nineteen and thirty-three.

Finkelstein found that during the period there is a concentric narrowing of the field of vision, beginning one, two, or three days beforehand, reaching the greatest intensity on the third or fourth day of menstruation, and gradually disappearing on the seventh or eighth day after its appearance. The narrowing of vision is more pronounced in those women in whom menstruation is associated with headache, cardiac palpitation, and other nervous symptoms, as well as in those who lose large quantities of blood.

Not only the field of vision for white, but also the visual fields for green, red, yellow and blue undergo a regular diminution. Perversion of perception of green (which is seen as yellow) is observed fairly often (in 20 per cent of all cases).

It would be fair to assume that careful observation of the other sense organs would yield similar proof of monthly periodicity.

Lennox Browne, a prominent specialist, wrote to Havelock Ellis, as follows, on the influence of normal menstruation on the voice: “With regard to singers, I believe that the pitch of the voice is often lowered at the menstrual epoch, although I have not found this to be universally admitted by patients to whom I have spoken on the subject. Many have told me that they have a disposition to sing flat, and in two cases in my recollection the patients, who suffered from dysmenorrhea (painful menstruation), told me that they sang sharp, of which they were conscious—probably from information, for of course you know that those who sing sharp are not generally sensible of the defect. It is, however, generally agreed that the timbre and tone-quality is impaired, the voice being decidedly thinner and poorer during the epoch.”

So well is this fluctuation of the voice recognized that in Continental Europe, it is not considered desirable for public singers to appear during the menstrual period. In fact, it is the custom on the Continent, when making engagements with female singers, to provide for the suspension of duty during the menses, but this does not apply in English or American contracts, although the **impresario** is in practice obliged to recognize it.

To bring the subject from the realm of art down to the more prosaic sphere of the household, Clouston thinks it probable that the superstitions regarding the evil influences exercised by women at their periods on foods, etc., which they prepare, are based on the actual decreased success in such operations at this time, due merely to a physiological decrease in energy and skill.

The organic character of menstruation is well sustained by these many widely diversified evidences of its manifestations. The incident of vicarious menstruation, however, throws a further interesting light on the subject.

Vicarious menstruation has been observed in cases in which, as a consequence of morbid conditions of the uterus, or of the organism as a whole, the menstrual flow has at the time of puberty either failed entirely to appear or has been exceedingly scanty. When conditions of this kind have prevailed, hemorrhages from other organs have, since ancient times, been witnessed and are regarded as vicarious menstruation.

As the congestion that occurs during menstruation is not limited to the genital organs, when the flow of blood from the uterus fails to occur, the organism seeks another outlet, in order to restore the disturbed equilibrium of blood distribution. Vicarious flowing of blood, in these instances, may take place from the mouth, the nose, the intestines, the anus, the gums, the breasts, the ears, the lungs, or even the eyes.

These evidences of true vicarious menstruation must not, of course, be confused with hemorrhages due to genuine organic disease, such as tuberculosis, gastric ulcer, etc.

Numerous authorities have collected data regarding this phenomenon. Puech found, in the cases he collected, that vicarious menstrual hemorrhage occurred from the stomach thirty-eight times, from the mammary glands twenty-five times, from the lungs twenty-four times, and from the nasal mucous membranes eighteen times. In all cases menstruation had been long in abeyance. The regular monthly recurrence of the vicarious discharge is often marked.

CHAPTER IV

THE MENOPAUSE—CHANGE OF LIFE

The menopause, or “change of life,” also called the climacteric period, represents the end of the reproductive life of woman. It does not imply, however, that this is the end of a woman’s sexual life—that is, the loss of sexual feeling and the capacity for sexual gratification. A further word will be said upon this subject later in the present chapter.

The change is usually not a sudden one, the symptoms gradually making their appearance, and the decline of the reproductive processes taking place slowly but continuously. The resultant disturbances, of course, involve the whole organism, affecting the functions of many of the organs, and causing other distressing reactions.

This epochal series of events is started in the first place by changes that occur in the ovaries. The tissue changes that take place in these organs produce a powerful ovarian stimulus. The normal internal secretion of the ovaries, characteristic of the female, which is responsible for the typical physiological attributes of the sex, is no longer contributed to the blood stream.

The specific manifestations of the menopause are, first, as the very name implies, the permanent termination of the menstrual flow; the occurrence of a number of nervous disturbances, disorders of circulation, tension of the blood vessels, and of the metabolism.

These manifestations are so striking that they have, from very remote times, led to the belief that they involved great danger to woman’s life at the climacteric period.

Available statistics, however, do not seem to lend much credence to this assumption. While it is a more or less trying period and the far-reaching changes in the woman’s organism are attended by certain grave possibilities, they are certainly not so dangerous as those involved in the high-tide of woman’s sexual life, due to pregnancy, parturition, and the after

effects of childbirth, not to mention the hazards of venereal infection in the early years of woman's active sexual life.

In this connection Kisch states: "It is often asserted that in this 'critical period' of the menopause, the mortality of the female sex is notably increased. The data available are somewhat conflicting, but a careful examination leads us to believe that, if due allowance is made for the natural increase in the mortality with advancing years, no important increase in the mortality of woman can be traced as due to the troubles and disturbances of the climacteric period."

This authoritative observation is very important, and should do much to reassure women approaching or undergoing the trials of the climacteric age.

AGE AT WHICH MENOPAUSE OCCURS

Like the onset of puberty and other factors that enter into the sexual life of woman, there is a wide range of variation in the appearance of the menopause. The usual time, in the temperate zone, is between the ages of 45 and 50, although it may occur before or after these ages. The normal limits have been mentioned as from 44 to 52; anything before or subsequent to those ages being considered exceptional.

The principal circumstances that seem to influence the occurrence of the menopause are the following:

1. The climate.
2. The race of the woman.
3. The age at which puberty occurred.
4. The sexual activity of the woman's life, including the number of pregnancies, and the use or non-use of the function of lactation.
5. The social and economic conditions of the woman's life.
6. General constitutional and pathological conditions, particularly from the standpoint of heredity.

Inasmuch as it had been noted that under climatic influences puberty and the menopause have a chronological relationship, it was once accepted as a fact that when puberty began early, the menopause would take place early;

and when puberty occurred late, the menopause was correspondingly retarded.

A more careful analysis of all the circumstances involved, however, indicates that this is true only in regard to the influence of climate upon sexual development. It is true that in a cold climate a woman begins to menstruate late and ceases to menstruate late, and that in a hot climate the opposite situation prevails.

However, if a comparison is made between women **living under similar conditions** with respect to latitude and climate, there is a very different result. On this basis, it is observed that the earlier in life a woman begins to menstruate, the later will be the age at which this function will cease.

The chief exception to this rule seems to be where menstruation has occurred at an **abnormally** early age, as, for instance, before the age of 12. Extremes either way, precocious menstruation, or when greatly retarded, usually work toward the same end—a premature menopause.

In women who enjoy good health, whose menstrual flow has always been regular and sufficient in quantity, whose sexual functions and reproductive powers have been adequately exercised, who have given birth to several children, and have suckled these children, the menopause generally takes place considerably later than in women whose sexual life has been comparatively inactive, or in whom the opposite conditions have prevailed than those previously mentioned.

The more regular menstruation has been, the more normal the deliveries, the later does the climacteric take place.

Women who have given birth to a number of children, as a rule menstruate for several years later than sterile women, and somewhat later than those who have had one or two children. The menopause also appears to be postponed, other things being equal, when a woman has nursed her children.

If there has been an excessive number of pregnancies, the menopause is likely to appear early. The same result occurs when sexual intercourse has taken place at too early an age.

Constitutional factors also are important in influencing the time of the menopause. Women who have inherited a weak or delicate constitution, in

whom the menstrual flow has always been pale or scanty, and the intermenstrual intervals unduly long, experience the climacteric earlier than strong, healthy women.

Generally speaking, women with a tendency to obesity cease to menstruate earlier than women of more normal build; blondes earlier than brunettes; women of phlegmatic temperament earlier than women of a sanguine and ardent temperament.

All those influences which have a weakening effect upon the feminine organism tend to hasten the advent of the menopause. Besides those mentioned in the preceding paragraph, factors such as severe labor, great sorrow, onerous occupations, abortions, local diseases of the reproductive organs, as well as distinctly pathological states, impel toward the same end.

VARIOUS MANIFESTATIONS OF APPROACH

The observations of gynecologists indicate that the duration of the climacteric manifestations varies largely between six months and three years. The mean duration, from the commencement of the symptoms until the cessation of menstruation, is about two years.

In a small proportion of cases the manifestations do not last over a month or two, while on the other extreme, cases are reported in which the duration has extended to four, six, eight, and even eighteen years (Kisch).

Tilt, a recognized English authority, has obtained some significant data about the various manifestations of approach of the menopause. Among a large number of women, sufficient to make a test of real value, he found the climacteric to occur as follows:

	Percentage of All Cases
By gradual diminution of the amount of flow	26.85
By irregular recurrence of menstruation, at intervals exceeding 21 days	15.54
By sudden interruption of the flow	14.76

By a terminal attack of metrorrhagia (uterine hemorrhage between menstrual period)	12.87
By a series of attacks of metrorrhagia	8.79
By sudden interruption and a terminal attack of metrorrhagia	6.75
By alternations of very profuse and very scanty menstruation	5.65
By irregular recurrence of menstruation, at intervals of less than 21 days	5.18
By irregular recurrence of menstruation, the intervals being sometimes longer and sometimes shorter than 21 days	3.61
	100.00

An interesting physiological observation is made from the extensive researches of Andral and Gavarret, who have shown that in the female sex the quantity of carbonic acid eliminated by the lungs diminishes when menstruation first appears at puberty, but increases again at the climacteric age, with the cessation of the menstrual flow.

In the male, on the other hand, a gradual diminution in the elimination of carbonic acid begins in the 36th year of life.

In old age the quantity of carbonic acid eliminated is greatly reduced in both sexes.

Many authorities believe that too-early marriage, sexual excesses, and prostitution are chief factors in a premature climacteric. In some instances, at least, there seems to be a hereditary predisposition in this respect, since it has been observed that the mothers of women in whom it has occurred early have themselves been similarly affected. Of course, it is possible in some cases of this kind that the same external and environmental factors which caused the phenomenon in the mother also produced the similar result in the

daughter—such, for instance, as sexual excesses, too-early marriage, or premature sexual experience.

The difficulties and disorders attending premature menopause are commonly more severe and more lasting than those that take place at the normal menopause. This is especially so if the premature menopause comes on quite suddenly, which, however, is comparatively rare.

The premature menopause usually approaches gradually. The flow becomes more scanty month by month, until finally it ceases altogether.

The premature climacteric may sometimes be delayed by expert gynecological treatment. Kisch refers to several cases in which the climacteric occurred at 35, 38, 39 and 42 years, respectively, when as a result of hydropathic treatment, regular menstruation recurred. In one case, a woman who had ceased to menstruate ten years before, gave birth to a child at the age of 45.

Retarded menopause presents some very interesting aspects, especially when late pregnancies have followed, many examples of which are on record. It sometimes happens that after the apparent termination of the menstrual periods, the flow recommences at an unusually late age.

Capwron quotes the case of a woman who became pregnant at the age of 65 years, with the recurrence of the menopause, which had ceased many years previously at the normal age. This woman aborted at three months, and the fetus was well formed.

Renaudin relates the case of a woman 60 years of age who gave birth to a child. Menstruation had ceased 12 years earlier.

A most remarkable instance is that reported by Meissner, of a woman who first began to menstruate at the age of 20. She had her first child when 47 years old, and gave birth to the last of her eight children in her sixtieth year.

The following are a few of the many authentic cases of protracted menstruation. Kisch reports no less than 106 cases in which the menopause did not occur until after fifty, of which five were as late as 57, two at 58, one at 59 and one at 60. Of Tilt's many cases, two were at 57, four at 58, one at 59, one at 60, and two at 61. Courty instances a case in which

menstruation persisted after the age of 65; Mayer, three cases at 64; Beigel, two cases, one at 65 and the other at 72.

To recur to the normal phase of the subject, as I have said, the actual dangers incident to the menopause have been much over-rated in the past. There are, however, two principal hazards which should be noted. First, there is the great tendency to profuse uterine hemorrhage, and, secondly, the liability to the occurrence of malignant tumors of the ovaries, uterus and mammary glands (breasts).

SEXUAL ACTIVITY AFTER THE CLIMACTERIC

Probably more distress and real suffering incident to the menopause is due to the mental attitude toward the phenomenon than is generally realized. A woman may have a feeling—as unquestionably many do—that her sexual life is over, that she is *passe*, and will no longer have the same attraction for her husband in particular, or the male sex in general.

The influence of a mental attitude of this kind is in itself quite pernicious. We know the relationship between ideas and physical states—autosuggestion—and how the former tend to realize themselves in the corporeal being. Not only that, but the mental and spiritual reactions are depressing, and conducive to morbidness and despondency.

Even the most philosophical woman, when under the influence of such negative ideas, has her outlook on life distorted. A woman with less mental control becomes embittered at her fate, which may lead to all sorts of nervous and psychological complications, with their unfavorable physical responses.

It would be interesting to know, if all the facts could be ascertained in a large number of cases of pathological menopause, what percentage of morbidity is really due to false notions and *unhealthy* idea-complexes. Much misery could be eliminated by some rational insight into these problems.

Men, of course, in their ignorance of sexual matters, are to a large extent responsible for perpetuating this condition. Edgar Selwyn's successful and somewhat sensational play, "*Dancing Mothers*," uses this situation as its central idea. *Ethel Westcourt*, having reached her forties, supposed herself

passe—in which supposition her philandering husband and flapper daughter fully concurred. Her husband had told her that the woman of forty is the dispassionate high-priestess of her sex. She decided to teach them a lesson by going out and pretending to live as lightly and loosely as they. In her first contact with the gay outside world, she discovered that she was not *passe*, that she still possessed charms that could successfully compete in the amorous marts, even with those of young girls of her daughter's age. By the irony of fate, she not only eclipsed her daughter in the eyes of one *Naughton*, a man about-town, but also her husband's promiscuous lady friend, an ex-mistress of *Naughton's*. So this "dispassionate high priestess of her sex" proved herself a woman in whom the human passions burned as brightly as in the heyday of passionate womanhood.

As a matter of fact, the idea behind this theme is more authentic than perhaps even the author realized in his search for dramatic material.

The *restlessness*, physical, mental and spiritual, of woman at this age is proverbial. Our shortcoming has been in failing to recognize the trouble. A diagnosis of the actual conditions would have automatically supplied its own remedy. We can successfully cope with a situation only when we have an intelligent understanding of it, and not until then.

Dr. William J. Robinson has discussed this question with characteristic wisdom in the following words: "The generally prevalent opinion among women, and men for that matter, and not only the laity, but unfortunately of the medical profession as well, is that the menopause is the end of a woman's sexual life. Nearly every woman labors under the erroneous impression that with the establishment of the menopause, with the cessation of the menses, she ceases to be a woman, and as she does not become a man, she becomes something of a neuter being, neither man nor woman. And she has the idea that after the menopause she can have no further attraction for her husband or for other men. Naturally, such an idea has a very depressing effect on any human being. A human being fights to the last to retain all its human functions, especially a function which is considered as important as the sexual function.

"Of course, with the permanent cessation of the menses, the woman's *reproductive* function is at an end. But the reproductive function is *not* synonymous with the sexual function, as I must insist again and again, and

naturally until this erroneous idea is dispelled much unnecessary misery will be the lot of our women. If women in general will learn that with the establishment of the menopause they do *not* cease to be women; if they will learn that the sexual desire in women lasts long beyond the cessation of the menopause, many women being as passionate at sixty as at thirty; if they will learn that their attractiveness or non-attractiveness to the male sex does not depend upon the menopause, but upon their general condition, if they will learn that many women at fifty or sixty are much more attractive than some women half that age, they will not take the onset of the menopause so tragically and they will thereby avoid the greater part of this mental and emotional suffering.”

CHAPTER V

SEXUAL BASIS OF NERVOUS DISORDERS

Nervous disorders are so widely prevalent in the life of modern woman that any serious attempt to throw light upon the subject is of decided service to the cause of health.

Within comparatively recent years there has been a growing tendency in various therapeutic circles to emphasize the relationship of nervous disturbances to the sexual sphere. Freud, the Viennese psychoanalyst, is probably more responsible than any other individual for this trend. However, the policy of recognizing the sexual basis of nervous disorders is not now by any means restricted to the Freudians, or even the dissenting schools of psychoanalysis. All psychiatrists, neurologists, and the medical profession generally, are coming more and more to a realization of the potency of sex in starting off all sorts of nervous ills.

Of course, it is not contended by the present writer that every case of nervous disorder has its roots in sex; nor that there are not sometimes other contributing factors involved. The human organism is so complex, and the interrelations of its component parts are so marked, that it is often exceedingly difficult to find the precise cause of a given group of symptoms.

The conception of woman as a mysterious, sexually anesthetic being is now largely a thing of the past. As Dr. Phyllis Blanchard remarks in *The Adolescent Girl*: "One of the most tantalizing problems is woman, because she has remained a baffling mystery, as shown by the folklore and literature of every people at every level of culture, in spite of her intimate relationship to man and the commonplaceness of her existence. In the attempt man has made to fathom her soul, she has been deified or endowed with demonic powers as the case may be."

Now that woman has jumped off the pedestal (which in theory only man had erected for her), and refuses to be classified as a demonic being in her

contrary moments and moods, we are free to consider her case rationally and scientifically as that of a genuine human being.

SEX FACTORS IN ADOLESCENCE

The yearning and stress incident to the awakening of the sexual life of the girl at puberty have very far-reaching effects, although normally there is less concentration upon the genital zone than is the case with the boy. But on account of the very fact of the lack of this concentration, there is a wider distribution of the emotional reactions.

In general, the sexual impulse in the young girl manifests itself in the form of reverie, hazy sensations, with presentiments that are apt to lead to melancholy moods. If circumstances bring her in contact with sexually experienced female friends, or other ardently erotic stimuli, the sexual impulse may be transformed into a hectic flame.

Any sort of emotional outlet that lies at hand may be grasped at the time of this upwelling of strange and conflicting forces. In former times the objective emotional interests were found principally in religion, poetry, music and esthetics. The religious zeal of the young woman at the adolescent period was formerly a very commonplace thing. It was something to which rich and poor alike had recourse to; whereas the girl of poor family was denied the opportunity for finding an emotional outlet in music, esthetics or poetry (her drab, prosaic surroundings precluded the latter no less than music and beauty.)

Nowadays, young women are actively identified with every phase of life—commercial, industrial, and otherwise, that offer opportunities for emotional outlet that were formerly undreamed of. In addition to these, of course, there is also the great personal freedom which girls and young women now enjoy and naturally, sometimes abuse.

The love of early youth, Krafft-Ebing remarked, has a romantic, idealizing tendency. In its first manifestations it is platonic, and willingly exercises itself in poetry and history. But as the sensibility awakens, the danger arises that this passion with its idealizing power, will be transferred to persons of the opposite sex who in intellectual, physical and social relations are by no means all that could be wished, hence proceed

misalliances, elopements and seductions, with the entire tragedy of impassioned love, which conflict with the dictates of morality and convention, and sometimes finds its bitter end in suicide or a double self-destruction. Love in which the senses play too prominent a part can never be a true and lasting love. For this reason, first-love is as a rule very transitory, since it is in most cases no more than the first flare of passion. Platonic love is a thing without existence, a self-deception, a false description of sexual sensations.

Bebel found that the number of suicides among women of the ages of sixteen to twenty-one years is an exceptionally large one, and he attributes this chiefly to unsatisfied sexual impulse, unfortunate love, secret pregnancy, and to betrayal by men.

Sexual emotion, tension or preparedness, as Dr. Frink has pointed out, is less dependent on external conditions than are the other normal emotions. We do not feel continual normal anger or fear unless we are continuously subject to an external danger. But sexual tension, or preparedness, may arise in the absence of any external stimulation, and tends to persist until temporarily relieved by some suitable action, of which coitus, in the adult, is normally the most satisfactory.

Thus, in the absence of actions adequate in quality or in frequency to discharge the tension, there may come about a state of organic sexual preparedness which is chronic—even though the individual is not continuously aware of sexual desire. In other words, a lack of adequate sexual outlet may result in the accumulation in the blood of abnormal quantities of thyroid secretion and perhaps, sugar, adrenin, and other substances which constitute also an important part of the state of preparedness for non-sexual exertion, such as attack or flight, and this very likely is accompanied by corresponding changes in the sympathetic nervous system.

DISORDERS DUE TO ABSTINENCE

Whereas in youth, abstinence is more easily borne by woman than man, at full maturity woman's sexual urge, as unconsciously manifested, if not, indeed, consciously—when not adequately gratified—may be quite as insistent as in man.

Nystrom states that abstinence has often as severe an effect upon woman as upon man, although some difference occurs in the manifestations, because of the difference in the physiological organization of the two sexes.

The consequences of prolonged self-denial, especially in women whose sexual desire has been thoroughly aroused by erotic experience, are much the same as in men. They show the same symptoms of functional disturbance, embracing a wide range of disorders.

The most common disturbances due to abstinence are chlorosis (a form of anemia), “female diseases,” nervousness in its many phases, cramps, hysterics with a tendency to alternate laughing and crying, hilarity and sadness, insomnia, hallucinations, mental disorders, etc.

Professor Erb, who has made important investigations in this domain, is under the impression that the health of many young women of good breeding and exemplary morals has suffered from abstinence. He considers that undoubtedly innumerable older, single women of refinement and education become sick or ailing from abstinence.

A number of married women who had been neglected by their husbands, and widows, have told him that they suffered severely from their enforced continence, and most of them became neurasthenic and hysterical. Nerve specialists generally, who so often come across these cases, have reported the same experience.

Numerous young, healthy, normal women related to Erb how, from the mid-twenties, they have become ill from semi-conscious sexual desires and ungratified craving for love, have experienced sexual fancies and hallucinations, melancholy and “female diseases,” etc.

Of course, there is a considerable number of women who are sexually anesthetic, or frigid, and appear to have no primary sexual feeling whatever. In true cases of this kind, they never experience the sexual urge, even after having borne children. Their genital organs are so insensible to feeling that they do not respond to artificial irritation, such as electricity, etc.

It is generally believed by sexologists and gynecologists, however, that a considerable proportion of cases of so-called frigidity is not organic or constitutional, but are due to long-established repression, fear of pregnancy and false, prudish ideas of sex—considering it “nasty.” They are the victims

of miseducation. Another not inconsiderable number has been improperly initiated by a blundering husband. Their first sexual experiences were rude, brutal awakenings which so revolted them that it established a chronic feeling of antipathy, or at best apathy, toward the sexual function. Some women never overcome this attitude, implanted by an ignorant or inconsiderate husband. In others, under the influence of love and kindness, and with the later acquisition of insight into sexual problems, there is gradually established the normal sexual feeling.

Dr. Hegerisch has written: "Although I agree with Malthus in appreciation of womanly virtue, I must as a physician express as my opinion that the chastity (abstinence) of woman nevertheless is a crime against nature, and often is followed by serious functional disturbances and symptoms of diseases."

Practically all the leading authorities in this field have supplemented their general comments by citing numerous cases, showing specifically the results of abstinence in normal, sexually mature women.

Occasionally, of course, we find brilliant, intellectual women living an active, productive life without apparently suffering from sexual abstinence.

Then there is another type of woman, full of energy, some of which is possibly transferred from the sexual goal, who interests herself in, or more literally throws herself into, various movements and organized activities, sometimes with remarkable constructive and consistent results, and at other times merely as a means of "expressing herself," without capacity for co-operation or tolerance for the ideas of others who show a dissenting opinion.

NEURASTHENIA

Freud makes the astounding statement that in a normally constituted sex life no neurosis is possible.

This is based on a vast experience in the realm of nervous and mental disorders. It is not my purpose to defend this opinion *in toto*, but to offer it as the thoughtful observation of the world's foremost medical psycho-neurologist.

If it has even a seventy-five per cent basis of fact to sustain it, it is still a remarkably striking recognition of the potency of sex in dominating our lives, even when we are not aware of the force that is tossing us hither and yon.

It must not be assumed that this statement of the influence of the sexual libido on the individual refers exclusively, or even in a major respect, to the present status of the person's sex life. It includes the entire range of sexual history of the individual, meaning every sex impulse, thought, act and experience, either in relation to another or others, or purely autosexual acts and experiences.

If the individual sex history (from infancy) has been sound and healthy, if fate has interposed no deleterious experiences, and, of course, if heredity has been kind and generous, then there is the foundation for a healthy sexual constitution.

Assuming that this brings us up to the portal of adulthood, in the present case, *womanhood*—there follows the obligation to lead a sex life that is in harmony with the normal impulses and at the same time with the ideals.

If the ideals are irrational, or if difficulties intervene and prevent the living of a sane sex life, even then upon the good foundation of sexual health, there may develop neurasthenic flaws in the superstructure of sexual disharmony.

In his classification of the neurasthenic symptom-complex, Freud maintains that the cause of attacks of anxiety-neurosis is very frequently to be found in a number of injurious influences in the sphere of sexual life. In women, these anxiety-neuroses occur:

1. As virginal anxiety, or anxiety of adolescents. This authority has observed a number of unequivocal instances showing that a first encounter with the sexual problem, a rather sudden unveiling of what has hitherto been concealed, as, for instance, the sight of some sexual object or act, or something read or heard in conversation, may, in a girl at the time of puberty, give rise to an anxiety-neurosis, which is in a very typical manner combined with hysteria.

(This is an excellent argument for the proper enlightenment of young people regarding the nature of sex and what the continuance of the race

implies. They have to find out sooner or later, and if the subject is treated by parents in a matter of fact manner, these vital facts of life are gradually and unobtrusively absorbed, without alarm, shock or stimulation.)

2. As anxiety of the newly married. Young wives who have been without sexual feeling in their first experience are not infrequently attacked by an anxiety-neurosis, which, however, disappears as soon as the sexual feeling becomes normal. Since, indeed, the majority of young women who lack sexual feeling in their first experience of sexual intercourse remain nevertheless quite healthy, it is evident that some other cause must co-operate in arousing the anxiety-neurosis.

3. An anxiety in married women whose husbands suffer from *ejaculatio praecox* (premature ejaculation) or from great diminution of sexual potency, or

4. Whose husbands practice *coitus interruptus*, or *coitus reservatus*. Cases in these two classes are closely associated, since it is easy to ascertain from the analysis of a sufficiently large number of cases that the really important question is, whether during coitus the wife obtains or fails to obtain sexual satisfaction. In the latter event, the condition requisite to arouse the anxiety-neurosis is supplied.

5. As anxiety in the climacteric period, during the termination of the reproductive life, which frequently involves a flare-up of passion.

Various anomalies of the genital organs which give rise in the virgin to no trouble whatever, display their influence during maturity by unfavorably affecting the nervous system. Thus, in cases of malformation of the external organs of reproduction, imperforation of the vaginal canal, a rudimentary condition of the vagina, rigid hymen, or local disturbances in the vagina, it is only when sexual intercourse begins that neuroses or hysteroneurasthenic troubles ensue.

In a paper setting forth his views on the etiology of the neuroses, Freud states: "I was above all surprised in the frequency of coarse disturbances in the sexual life of patients. The more I was in quest of disturbances, during which I remembered that all men conceal the truth in things sexual, and the more skilful I became in continuing the examination despite the incipient negation, the more regularly such disease-forming moments were

discovered in the sexual life, until it seemed to me that they were but little short of universal.”

It was Freud’s particular experience in this field, even comparatively early in life which led him to the conclusion that “in a normal sex life no neurosis is possible.”

Years after he originally made this statement, he was confirmed in its correctness, although he had passed a good way beyond his former point of view in his opinions concerning the relation of sexual traumas to health in general. This gave him a deeper insight into the problems of sex in their universal application.

For instance, by chance his former material furnished him with a great number of cases of infantile histories in which sexual seduction by grown-up persons or older children played the main role. He therefore over-emphasized the frequency of these occurrences (otherwise not to be doubted), the more so because he was then in no position to distinguish definitely the *deceptive memories* of hysterical patients, whereas he subsequently learned to explain many a seduction fancy as a defense against the reminiscence of the patient’s own sexual activity (infantile masturbation).

“The emphasis laid on the ‘traumatic’ (wounding) element of the infantile sexual experience disappeared with this explanation, and it remained obvious that the infantile sexual activities (be they spontaneous or provoked), *dictated the course of the later sexual life after maturity.*”

Of course, there is an almost immeasurably wide range of variations in the psychic and nervous constitution of individuals—ranging for our present purpose—from the ultra-sensitive to those of extremely low sensibility. Those persons of the former type, being most *impressionable*, are more readily influenced by psychic traumas or soul wounds at any time of life.

The opposite types (the “tough-minded” of William James’ classification), are less susceptible to psychic wounds and hurts, and therefore do not so readily become victims of the various neuroses.

No group of human beings, however, is exempt from the possibility of neurasthenic troubles, if conditions are favorable for their development.

And no ground is so propitious a breeding place for these disturbances as the fertile soil of sex under the highly artificial development of modern civilization, and the confusing ideas that prevail upon the vital question of sex ethics.

Transcriber's Note:

Inconsistencies in hyphenation have been standardized.

Minor punctuation errors have been changed without notice.

The [table](#) on pp. 26-27 originally spanned across the two pages horizontally. This could not be recreated in the HTML version, so it was split between two pages.

Spelling was retained as in the original except for the following changes:

Page 4 : “presence of abnormal erotism”	to “presence of abnormal eroticism”
Page 5 : “over the calibre or”	to “over the calibre of”
Page 8 : “lost in the magnificant”	to “lost in the magnificent”
Page 10 : “Deutschlands litt und rel. Verhaltn in Reformations zeit”	“Deutschlands lit. u. relig. Verhältnisse im Reformationszeitalter”
Page 18 : “the first menstruation occuring”	to “the first menstruation occurring”
Page 25 : “days, one way of the other”	to “days, one way or the other”
Page 25 : “demands of the embryo”	to “demands of the embryo”
Page 27 : “22 22 24” under “Jan.”	to “22 23 24”
Page 30 : “Indian book of Susruta”	to “Indian book of Suśruta”
Page 31 : “and in the rigid”	to “and in the frigid”
Page 32 : “well-to-do, whose modes”	to “well-to-do, whose mode”
Page 37 : “the head, epileptic paroxysms”	to “the head, epileptic paroxysms”
Page 39 : “of the household, Clauston”	to “of the household, Clouston”
Page 42 : “and of the metabulism”	to “and of the metabolism”
Page 42 : “after effects of childb-irth”	to “after effects of childbirth”
Page 45 : “has been, the moral”	to “has been, the more”

Page 47 : “researches of Andral and Gavaret”	to “researches of Andral and Gavarret”
Page 49 : “at late as 57”	to “as late as 57”
Page 50 : “this condition. Edgar Selwin’s”	to “this condition. Edgar Selwyn’s”
Page 51 : “concurred. Her husband has”	to “concurred. Her husband had”
Page 51 : “brightly as in the heyday”	to “brightly as in the heyday”
Page 56 : “proceed misalliances, elopements”	to “proceed misalliances, elopements”
Page 62 : “suffer from <i>ejaculatio precox</i> ”	to “suffer from <i>ejaculatio praecox</i> ”

*** END OF THE PROJECT GUTENBERG EBOOK WOMAN'S
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